

FOREIGN TOMO BREAST BILATERAL

Status: Edited Result - FINAL

PACS Images

(Link Unavailable) Show images for FOREIGN TOMO BREAST BILATERAL

Addendum

ADDENDUM:

EXAM: REVIEW OF SUBMITTED BREAST IMAGING:

BILATERAL MAMMOGRAM WITH TOMOSYNTHESIS August 28, 2025:
Technique: bilateral mammogram: tomosynthesis images were obtained.

RIGHT LIMITED BREAST ULTRASOUND August 20, 2025:
Technique: right breast us: limited breast ultrasound was performed.

Electronically Signed By: TALI AMIR MD on 3:27 PM

Addended by Tali Amir, MD on 3:27 PM

Study Result

Narrative & Impression
ACCESSION#(s): A000224096, A000224098

DATE: 12:33 PM

EXAM: REVIEW OF SUBMITTED BREAST IMAGING:

BILATERAL MAMMOGRAM WITH TOMOSYNTHESIS August 28, 2025:
Technique: bilateral mammogram: tomosynthesis images were obtained.

RIGHT LIMITED BREAST ULTRASOUND August 20, 2025:
Technique: bilateral breast us: limited breast ultrasound was performed.

CLINICAL HISTORY: Newly diagnosed palpable right breast cancer.

COMPARISON(S): None

FINDINGS:

MAMMOGRAM:

BREAST DENSITY: Heterogeneously dense, which may obscure small masses.

RIGHT BREAST:
Subjacent to a BB denoting site of patient's palpable lump upper inner breast, 2:00 axis posterior depth there is a spiculated mass with associated architectural distortion. Associated calcifications appear to extend beyond the mass, not evaluated with magnification views. There is diffuse skin thickening of the lower breast, involving the periareolar. There is slight nipple retraction.

A BB is noted overlying the right axilla. Palpable lymph nodes on ultrasound outside the field-of-view on mammogram.

LEFT BREAST:
Left outer central breast posterior depth there is a 1.4 cm asymmetry (CC 36/73; MLO 10/71).

ULTRASOUND: Evaluation is limited as interpretation is based on static images.

RIGHT AXILLA: Labeled palpable in the right axilla there is a 3.5 x 1.2 cm lymph node with nodular cortical thickening to 0.7 cm. Ultrasound-guided core biopsy was performed with needle seen traversing the lymph node. Post procedure mammogram demonstrates a vision marker in expected position. Outside pathology was yielded metastatic carcinoma.

FOREIGN TOMO BREAST BILATERAL

Order:

Status: Edited Result - FINAL Next appt: at 12:00 PM in Breast Surgery (Audree Blythe Tadros, MD)

Test Result Released: Yes (not seen)

Details

Reading Physician
Tali Amir, MD

Routine

Addenda

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EXAM: REVIEW OF SUBMITTED BREAST IMAGING:

BILATERAL MAMMOGRAM WITH TOMOSYNTHESIS August 28, 2025:
Technique: bilateral mammogram: tomosynthesis images were obtained.

RIGHT LIMITED BREAST ULTRASOUND August 20, 2025:
Technique: right breast us: limited breast ultrasound was performed.

Electronically Signed By: TALI AMIR MD on 3:27 PM

Signed by Tali Amir, MD on 15:27
Narrative & Impression
ACCESSION#(s): A000224096, A000224098

DATE: 12:33 PM

EXAM: REVIEW OF SUBMITTED BREAST IMAGING:

BILATERAL MAMMOGRAM WITH TOMOSYNTHESIS August 28, 2025:
Technique: bilateral mammogram: tomosynthesis images were obtained.

RIGHT LIMITED BREAST ULTRASOUND August 20, 2025:
Technique: bilateral breast us: limited breast ultrasound was performed.

CLINICAL HISTORY: Newly diagnosed palpable right breast cancer.

COMPARISON(S): None

FINDINGS:

MAMMOGRAM:

BREAST DENSITY: Heterogeneously dense, which may obscure small masses.

RIGHT BREAST:
Subjacent to a BB denoting site of patient's palpable lump upper inner breast, 2:00 axis posterior depth there is a spiculated mass with associated architectural distortion. Associated calcifications appear to extend beyond the mass, not evaluated with magnification views. There is diffuse skin thickening of the lower breast, involving the periareolar. There is slight nipple retraction.

A BB is noted overlying the right axilla. Palpable lymph nodes on ultrasound outside the field-of-view on mammogram.

LEFT BREAST:
Left outer central breast posterior depth there is a 1.4 cm asymmetry (CC 36/73; MLO 10/71).

ULTRASOUND: Evaluation is limited as interpretation is based on static images.

RIGHT BREAST: Labeled palpable, 2:00 axis III cm from the nipple there is a 6.0 x 2.5 x 4.0 cm irregular hypoechoic mass with indistinct margins, containing calcifications. Sonographic core biopsy was performed (limited needle position evaluation on submitted images). Post procedure mammogram demonstrates a coil marker in expected position. Outside pathology yielded invasive carcinoma.

RIGHT AXILLA: Labeled palpable in the right axilla there is a 3.5 x 1.2 cm lymph node with nodular cortical thickening to 0.7 cm. Ultrasound-guided core biopsy was performed with needle seen traversing the lymph node. Post procedure mammogram demonstrates a vision marker in expected position. Outside pathology was yielded metastatic carcinoma.

ADDITIONAL IMAGING RECOMMENDED:

- 1. Right breast magnification views as surgically warranted
- 2. Left breast mammogram and possible subsequent ultrasound. If MRI is planned the queried asymmetry may be further evaluated on MRI in lieu of mammogram and ultrasound

IMPRESSION:

- 1. Biopsy-proven palpable right breast cancer at the 2:00 axis with coil marker in expected position. MSK pathology review is pending. There is skin thickening of the lower right breast, areola and mild nipple retraction, suspicious. If it will alter surgical management, right breast magnification views are recommended to fully delineate extent of calcifications.
- 2. Metastatic right axillary lymphadenopathy with associated vision marker. MSK pathology review is pending.
- 3. Left breast asymmetry. Recommend left breast mammogram and possible subsequent ultrasound. If MRI is planned the queried asymmetry may be further evaluated on MRI in lieu of mammogram and ultrasound.

Electronically Signed By: TALI AMIR MD on 5:48 PM

Exam Ended: 12:33 Last Resulted: 15:27

-  Order Details
-  View Encounter
-  Lab and Collection Details
-  Routing
-  Result History - Result Edited

Result Care Coordination

 Patient

 Released  Not seen

Study Details

Open Study Details